

02-total-knee-arthroplasty-policy

Payer Prior Authorization Policy: Total Knee Arthroplasty (Unilateral, Primary)

CPT Code: 27447 **Applicable ICD-10 codes:** M17.11 (Unilateral primary osteoarthritis, right knee), M17.12 (left knee), M17.9 (Osteoarthritis of knee, unspecified) **Policy Number:** ORTHO-TKA-007 **Effective Date:** 2026-01-01

Purpose

Defines clinical criteria for prior authorization of primary unilateral total knee arthroplasty (TKA) for adult members with knee osteoarthritis.

Required Supporting Documentation

1. **Radiographic evidence of advanced osteoarthritis** — A weight-bearing knee X-ray report, performed within the 12 months preceding the request, documenting Kellgren-Lawrence grade 3 or 4 changes (joint space narrowing, osteophytes, subchondral sclerosis) OR an equivalent radiologist narrative describing severe/end-stage tricompartmental or unicompartmental osteoarthritis.
2. **Conservative management trial** — Clinical notes documenting at least 3 consecutive months of conservative management, which must include AT LEAST TWO of: NSAID or analgesic trial, structured physical therapy, activity/weight-bearing modification, and intra-articular corticosteroid or viscosupplementation injection — with dates and specific modalities.
3. **Documented functional limitation** — A functional assessment (WOMAC score, KOOS score, or a detailed narrative) documenting how knee pain limits activities of daily living (e.g., walking distance, stair negotiation, sleep disruption).
4. **BMI documentation and optimization plan if applicable** — Most recent BMI on file. If BMI ≥ 40 , the request must also include a documented weight-optimization or bariatric referral plan; BMI < 40 requires no additional documentation on this point.
5. **Absence of active infection or contraindication** — A note affirmatively stating the knee is free of active infection, and the patient has no absolute surgical contraindication (e.g., uncontrolled systemic infection, inadequate vascular supply).
6. **Pre-operative medical clearance** — For members over age 60 or with a documented cardiac, pulmonary, or diabetic history, a medical clearance / pre-operative risk assessment note from the primary care physician or relevant specialist, dated within 60 days of the request.
7. **Severe-presentation exception** — If conservative management was not completed for the full 3 months, the request may instead include documentation of a severe presentation justifying expedited surgery: radiographically confirmed bone-on-bone contact WITH clinical instability or a locked/mechanically blocked knee. When invoked, item 2 is not required, but items 1, 3, 5, and 6 remain required.

Exclusions

- Requests for bilateral simultaneous TKA require a separate policy (ORTHO-TKA-008) and are not covered by this document.
- Revision arthroplasty is out of scope for this policy.